

SKIN & LASER

Microneedling & RF Microneedling

Depth mapping, single-use cartridge policy, approved topicals and complication response

DOCUMENT SKN-003	VERSION 1.0	EFFECTIVE January 2026	REVIEW CYCLE Annual
APPLIES TO All microneedling operators	OWNER Medical Director	CLASSIFICATION Confidential	SUPERSEDES All prior versions

1 Purpose & Scope

This protocol covers both mechanical microneedling and radiofrequency microneedling. The two share depth and sterility requirements; RF adds thermal injury risk and a different complication profile. It also governs what may be applied to the skin during treatment — the area where the most serious avoidable harm occurs.

YOU ARE CREATING THOUSANDS OF OPEN CHANNELS

Microneedling breaches the epidermal barrier deliberately and repeatedly. Anything on the skin surface goes into the dermis. Non-sterile products, cosmetic serums and preservatives that are entirely safe applied topically can cause foreign body granulomas, prolonged inflammation and permanent change when driven into the skin.

2 Screening & Exclusions

- ☐ Active acne with inflammatory or pustular lesions in the field — defer; needling spreads bacteria through the channels.
- ☐ Active infection of any kind, including warts and impetigo.
- ☐ Herpes simplex history where treating perioral areas — prophylaxis per the medical director.
- ☐ History of keloid or hypertrophic scarring.
- ☐ Isotretinoin within the interval set by the medical director.
- ☐ Anticoagulant therapy or bleeding disorder — assess and document.
- ☐ Immunosuppression, poorly controlled diabetes, or impaired healing.
- ☐ Pregnancy — practice policy set by the medical director.
- ☐ Recent laser, peel or injectable in the same area within the defined interval.
- ☐ Undiagnosed or changing pigmented lesions in the field — do not needle over them.
- ☐ Fitzpatrick documented; higher types carry greater post-inflammatory hyperpigmentation risk and warrant conservative depth.
- ☐ Metal allergy, pacemaker or implanted electronic device — critical for RF platforms.

3 Depth Selection

Depth is selected by anatomical zone and by indication, and reduced for thinner skin and for higher Fitzpatrick types. The mapping below is a framework; your medical director sets the practice ranges against your device IFU.

FRAMEWORK BY ZONE

Zone	Relative depth	Note
Periorbital and around the nose	Shallowest	Very thin skin. Use the minimum effective depth and reduce speed.
Forehead and temples	Shallow	Thin skin over bone; avoid excessive pressure.
Cheeks	Moderate	The thickest facial skin; tolerates the most depth.
Upper lip and chin	Moderate, reduced	Watch for pinpoint bleeding as an endpoint.
Neck and décolletage	Shallow	Thin skin, slower healing, higher risk of tram-tracking and prolonged erythema.
Acne scarring, body and stretch marks	Deepest, within IFU limits	Targeted rather than full-field. Requires explicit authorization.

ENDPOINT, NOT DEPTH NUMBER

The desired endpoint is uniform erythema with pinpoint bleeding. Aggressive bleeding, streaking or gouging means depth or pressure is excessive — reduce immediately. Chasing a depth setting rather than watching the skin is how tram-track scarring happens.

4 Sterility & Approved Topicals

- ☐ One sterile, single-use cartridge per patient. Never reused, never resharpened, never shared between patients under any circumstances.
- ☐ Cartridge opened in front of the patient where practical, and the packaging retained until the treatment is charted.
- ☐ Device body sleeved or barrier-protected and disinfected between patients per IFU.
- ☐ Sterile gloves and aseptic technique throughout.
- ☐ Skin prepped with an appropriate antiseptic and allowed to dry.
- ☐ Only products explicitly approved by the medical director applied during treatment.
- ☐ Used cartridges disposed of as sharps immediately.

Category	Position
Sterile hyaluronic acid preparations intended for this use	Generally acceptable. Confirm the specific product with the medical director.
Sterile saline	Acceptable as a glide medium.
Platelet-rich plasma	Acceptable where the practice has a validated PRP protocol and the operator is authorized. Autologous handling standards apply.
Cosmetic serums, vitamin C, retail skincare	Never. Not sterile, not formulated for intradermal delivery, and a documented cause of granuloma formation.
Products containing preservatives, fragrance or essential oils	Never. Preservatives are a specific granuloma risk in the dermis.
Topical anesthetic	Removed completely before needling begins. Cross-reference the dose limits in the Emergency Protocols kit (LAST).
Anything not on the approved list	Declined. Including products the patient brings in themselves.

PATIENT-SUPPLIED PRODUCTS

Patients will ask you to needle in a serum they bought. The answer is no, every time, and the reason is explained rather than negotiated. Record the request and the refusal.

5 RF Microneedling Specifics

- Insulated needles deliver energy at the tip and spare the epidermis; non-insulated needles deliver along the shaft. Know which your device uses — it changes the risk profile and the appropriate depth.
- Energy level, pulse duration and depth are set independently; confirm all three against the IFU for each pass and each zone.
- Epidermal cooling or contact cooling used as directed by the IFU.
- Absolute contraindications: pacemaker, implanted defibrillator, or other implanted electronic device. Screen explicitly and record.
- Metal implants in the treatment field require medical director assessment.
- Thermal endpoint is uniform erythema with mild edema. Immediate whitening, blistering or a burning odor means stop.
- Fat atrophy is a recognized risk with excessive energy over thin subcutaneous areas, particularly the temples and lower face — this is not reversible and warrants conservative settings.
- Downtime is longer than mechanical needling; set expectations accordingly.

6 Complications & Aftercare

Complication	Recognition	Response
Tram-track scarring	Linear marks following the pass direction, appearing over weeks.	Excessive depth, pressure or dragging. Photograph, notify the medical director, no further treatment pending review.
Granuloma	Persistent firm papules or nodules at treated sites, often weeks later.	Almost always a product driven into the dermis. Photograph, medical director notification, dermatology referral. Review what was applied.
Post-inflammatory hyperpigmentation	Darkening of treated areas, more common in Fitzpatrick III–VI.	Follow the PIH pathway in SKN-009. Strict sun protection.
Infection	Increasing pain, warmth, purulence, spreading erythema, fever.	Same-day medical assessment. Culture if discharge present. Antibiotics per prescriber.
HSV reactivation	Grouped vesicles, tingling, burning, usually perioral.	Antiviral treatment promptly. Review prophylaxis policy for future sessions.
Prolonged erythema	Redness persisting well beyond expected downtime.	Assess for excessive treatment. Bland care, sun protection, medical director review.

- Bland cleanser and emollient only until the barrier recovers; no actives.
- No makeup for the interval specified in aftercare — brushes and sponges are a contamination route into open channels.
- Strict sun protection; heat, sweat and exercise avoided for the specified period.
- No picking or exfoliating.
- Written warning signs supplied with same-day contact instructions.

7 Quick Reference

QUICK REFERENCE CARD

Microneedling non-negotiables

1. **New sterile cartridge, every patient.** No exceptions, ever.
2. **Approved sterile products only.** No retail serums. No patient-supplied products.
3. **Remove all topical anesthetic** before needling begins.
4. **Depth by zone**, reduced for thin skin and higher Fitzpatrick types.
5. **Endpoint = uniform erythema, pinpoint bleeding.** Streaking → reduce now.
6. **RF: screen for pacemakers and implanted devices.** Absolute contraindication.
7. **Active acne or infection in the field → defer.**
8. **Nodules weeks later = granuloma.** Photograph, escalate, review the product used.

Print, laminate and post at the point of care

8 Medical Director Authorization

This protocol is adopted as a standing order for the practice named below. By signing, the medical director confirms the protocol has been reviewed against current clinical guidance, applicable state scope-of-practice rules, and the training level of the staff authorized to perform it.

PRACTICE NAME	LOCATION / SITE
MEDICAL DIRECTOR — PRINTED NAME & CREDENTIALS	LICENSE NUMBER & STATE
SIGNATURE	DATE ADOPTED / NEXT REVIEW DATE

Template notice. This document is an editable template provided for educational and operational guidance. It is not legal advice, does not constitute a physician–patient relationship, and does not guarantee regulatory compliance or the outcome of any inspection. Drug doses, concentrations and device settings must be verified against current manufacturer labeling and prevailing clinical guidance before use. A licensed medical director must review, customize and sign this protocol before it is placed into service, and must confirm that every task assigned within it falls inside the scope of practice of the personnel performing it in your state.

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